

NCT07563010: Menin-Inhibitor Targeted Maintenance in AML

Gender: ALL

Age: 18 Years - MISSING

Criteria:

Inclusion Criteria:

Aged ≥ 18 years at the time of signing informed consent Able to provide written informed consent personally or via a legally authorized representative in accordance with applicable regulatory and institutional requirements Willing and able to comply with all study procedures and available for the duration of the study

Diagnosis of acute myeloid leukemia (AML) in complete morphologic remission with one of the following molecular abnormalities:

KMT2A-rearranged (KMT2Ar) AML Excluding KMT2A partial tandem duplication (KMT2A-PTD) NPM1-mutated (NPM1m) AML Including FLT3-ITD or TKD co-mutation NUP98-rearranged (NUP98r) AML Planned first allogeneic hematopoietic cell transplantation (allo-HCT) for AML. Transplant Characteristics Planned allo-HCT using bone marrow or peripheral blood stem cell graft source. Planned reduced-intensity/non-myeloablative conditioning (RIC/NMA) or myeloablative conditioning (MAC), using a conditioning regimen permitted- by the protocol and consistent with standard clinical practice, meeting CIBMTR criteria for conditioning intensity

Planned donor:

HLA-matched related donor (5/6 or 6/6) Matched unrelated donor (8/8) Mismatched unrelated donor (7/8) Haploidentical donor meeting institutional requirements

Performance Status:

Karnofsky Performance Status $\geq 70\%$.

Left ventricular ejection fraction (LVEF) by transthoracic echocardiogram (TTE) or multigated acquisition (MUGA) with no clinical evidence of heart failure:

RIC/NMA: $\geq 50\%$ MAC: ≥ 5 Pulmonary Function

Pulmonary function meeting the following criteria, without supplemental oxygen other than CPAP:

RIC/NMA: DLCO (corrected for hemoglobin) and FEV1 $\geq 40\%$ predicted MAC: DLCO and FEV1 $\geq 50\%$ predicted Estimated creatinine clearance (CrCl) $\geq 45\text{mL/min}$ calculated using the Cockcroft-Gault formula or 24-hour urine collection, consistent with standard eligibility criteria for allogeneic HCT recipients.

Liver function acceptable per local institutional guidelines for allo-HCT eligibility.

Reproductive Status: Willingness to use contraception in accordance with local regulations from first study intervention through the required contraceptive period Willingness to use contraception in accordance with local regulations from first study intervention through the required contraceptive period

Exclusion Criteria:

Disease Status Evidence of active AML prior to HCT, assessed within 42 days before transplant, defined as any of the following:

* 5% bone marrow blasts Circulating blasts within 14 days before conditioning CNS or other extramedullary disease Other active malignancy that, in the investigator's judgment, could interfere with safety or efficacy assessment Treatment with non-protocol antileukemic therapy (donor lymphocyte infusion for relapse prophylaxis or treatment will be considered an EFS event) Cardiac / QT Risk Requirement for concomitant medications known to prolong QT/QTc interval, except low-risk agents used as standard supportive care Diagnosis or suspicion of Long QT syndrome, or a family history of Long QT syndrome QTcF ≥ 450 msec.

History within 6 months of study entry of:

Myocardial infarction Unstable angina Congestive heart failure (NYHA Class $\geq$ II) Life-threatening or uncontrolled arrhythmia Cerebrovascular accident or transient ischemic attack Chronic respiratory disease requiring continuous supplemental oxygen, or other significant organ dysfunction that would adversely affect study participation.

Active, uncontrolled infection, including any of the following:

Active, uncontrolled systemic fungal, bacterial, or viral infection within 14 days prior to the start of conditioning Any other documented active, uncontrolled infection at the start of conditioning

Chronic viral infections with evidence of active disease, including:

HIV: detectable viral load within 6 months prior to screening

Hepatitis B:

HBsAg-positive and/or anti-HBc-positive with detectable HBV DNA, or Anti-HBc-positive alone (HBsAg-negative) with detectable HBV DNA Hepatitis C: positive HCV antibody with detectable HCV RNA Planned HCT using cord blood, ex vivo T cell depletion, engineered grafts, or experimental graft sources\\

Malabsorption syndrome or GI condition that precludes oral administration, including:

Inability to swallow oral medications Prior gastric bypass or severe gastroparesis Cirrhosis with Child-Pugh Class B or C Pregnant or breastfeeding Prior intolerance to menin inhibitor therapy resulting in $\geq$ Grade 3 treatment-related adverse events Any condition, therapy, laboratory abnormality, or allergy to excipients that, in the investigator's judgment, could confound study results, interfere with the participant's ability to comply with study procedures or complete the study, or make participation not in the participant's best interest.